

Healthcare and the Union Budget: Priorities for a Sustainable System

Healthcare rarely features in budget discussions as an economic growth engine. It is usually framed as a social obligation or a welfare expense. That framing is outdated. Healthcare today sits at the intersection of employment, infrastructure, technology, and productivity. As the Union Budget approaches, the question is not whether healthcare deserves higher allocation, but whether policy choices recognise its role in sustaining India's long-term economic momentum.

India has steadily increased public health spending to just over 2% of GDP. This is progress, but spending alone does not correct structural inefficiencies. Healthcare delivery remains expensive because care often begins too late and too far downstream. Weak preventive and primary care pushes patients into hospitals for conditions that could have been managed earlier and at lower cost. Budgetary focus on diagnostics, screening, and early intervention, particularly for non-communicable diseases, would not only improve outcomes but also release economic value by reducing avoidable hospitalisation and productivity loss.

Hospital economics reflect another quiet stress. Input costs, manpower, technology, energy, and compliance, have risen sharply while pricing mechanisms remain largely static. This imbalance is most visible in the mid-sized hospitals that serve large populations outside metros. These institutions are critical for access, employment, and regional development, yet they operate with limited financial buffers. Schemes such as Ayushman Bharat have expanded coverage, but their sustainability depends on periodic rate recalibration and predictable



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reimbursement cycles. Without this, financial strain accumulates invisibly until capacity expansion slows or quality erodes.

Healthcare's workforce challenge is now a macroeconomic issue. The sector is one of India's largest employers, yet shortages of nurses, technicians, and allied professionals continue to constrain growth. Burnout and attrition are not just operational concerns; they affect service quality, patient experience, and system throughput. Budgetary investments in skill development, structured career pathways, and workforce retention deliver returns that extend beyond healthcare, given the sector's employment intensity and multiplier effect.

Digital health is often discussed in aspirational terms, but its real economic value lies in efficiency. Poorly integrated systems increase administrative overhead and clinician fatigue. Well-designed digital adoption reduces errors, speeds decision-making, and improves asset utilisation. National digital health initiatives have laid a foundation, but

the next phase must focus on adoption across small and mid-sized institutions. Incentivising interoperable systems, cybersecurity readiness, and electronic records is not innovation theatre; it is productivity reform.

Healthcare manufacturing and medical devices also warrant a more economic lens. Import dependence raises treatment costs and exposes the system to supply shocks. Supporting domestic manufacturing, especially mid-scale players and component ecosystems, aligns healthcare affordability with industrial policy. This is not about protectionism, but resilience. Over time, it can lower costs, create skilled jobs, and strengthen India's position in global health supply chains.

Infrastructure investment in healthcare has slowed, largely due to expensive capital and long project timelines. Hospitals are long-life assets that require patient financing. Access to long-tenure capital and regulatory predictability would unlock private investment without compromising affordability. This is particularly important as India prepares for an aging population and rising demand for chronic, rehabilitative, and elder care services.

Healthcare policy compounds over time. The cost of inaction is rarely immediate, but it is always cumulative, showing up later as capacity shortages, higher costs, and lost productivity. A Budget that quietly corrects structural frictions will do more for India's economic future than one focused on announcements alone. If healthcare is treated as productive infrastructure rather than residual spending, the returns, economic and social, will be substantial.